

Moderate and Deep Sedation Policy

Pre-Sedation Assessment, Monitoring, and Recovery
Standards

Regulation: 42 CFR §482.52 | §482.51

POLICY OVERVIEW

Moderate and deep sedation administered outside the operating room carry significant risks of respiratory depression, airway compromise, and cardiovascular events. A comprehensive sedation policy ensures qualified personnel, appropriate monitoring, emergency preparedness, and safe recovery for all patients receiving procedural sedation.

KEY REQUIREMENTS

1 Pre-Sedation Assessment

A qualified practitioner must perform and document a pre-sedation assessment including: NPO status, relevant medical history, medication/allergy review, airway assessment (Mallampati score), ASA physical status classification, and consent for sedation.

2 Qualified Personnel

Moderate sedation must be administered or supervised by a practitioner with documented competency in sedation and airway management. A separate individual, other than the one performing the procedure, must monitor the patient throughout.

3 Intraoperative Monitoring

Continuous monitoring must include: pulse oximetry, capnography (end-tidal CO₂), cardiac rhythm, blood pressure (minimum every 5 minutes), and level of consciousness using a validated scale (Ramsay, MOAA/S). Findings must be documented on a sedation record.

4 Rescue Capabilities

All sedation locations must have immediately available: supplemental oxygen, suction, bag-valve-mask, resuscitation equipment, and reversal agents (flumazenil for benzodiazepines, naloxone for opioids). Personnel must be trained in rescue ventilation and CPR.

5 Post-Sedation Recovery

Patients must recover in a monitored setting until meeting defined discharge criteria (modified Aldrete score ≥ 9 or equivalent). Discharge criteria, monitoring intervals during recovery, and discharge instructions must be documented.

COMPLIANCE NOTES

The anesthesia department must review and approve sedation policies and credentialing criteria. Practitioners providing deep sedation must meet the same standards as those providing anesthesia. Adverse sedation events must be reviewed by the quality committee.

COMMON SURVEY CITATIONS

Cited under §482.52(a) – Anesthesia services organization, and §482.52(b) – Policies and procedures for anesthesia/sedation.